Suspected Acute Coronary Syndrome

Early Non-Invasive Testing after ED Visit

BACKGROUND

Professional guidelines recommend non-invasive testing (NIT) within 72 hours of an ED evaluation for suspected acute coronary syndrome (ACS).

QUESTION

Is early cardiac NIT after an ED evaluation for suspected ACS more effective than not testing?

OBJECTIVE

Evaluate the effectiveness of early NIT in reducing risk of death or acute myocardial infarction (AMI) within 30 days.

KEY RESULTS

- The absolute risk of death or myocardial infarction (MI) was low (**<1%**).
- Early NIT had minor benefit in reducing death (**0-2% (95% CI -0·2% to -0·1%)**); MI (**-0·3% (-0·5% to -0·1%)**); and major adverse cardiac events (MACE) (**-0·5% (-0·7% to -0·3%)**).
- The number needed to treat (NNT) was **250** to avoid one death or MI, **500** to avoid one death, **333** to avoid one MI, and **200** to avoid one MACE within 30 days
- Early NIT was associated with a **small decrease** in the risk of death or MI in ED patients with suspected ACS, but this clinical strategy **may not** be optimal for most patients given the large NNT

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Early Non-Invasive Cardiac Testing after Emergency Department Evaluation for Suspected Acute Coronary Syndrome

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